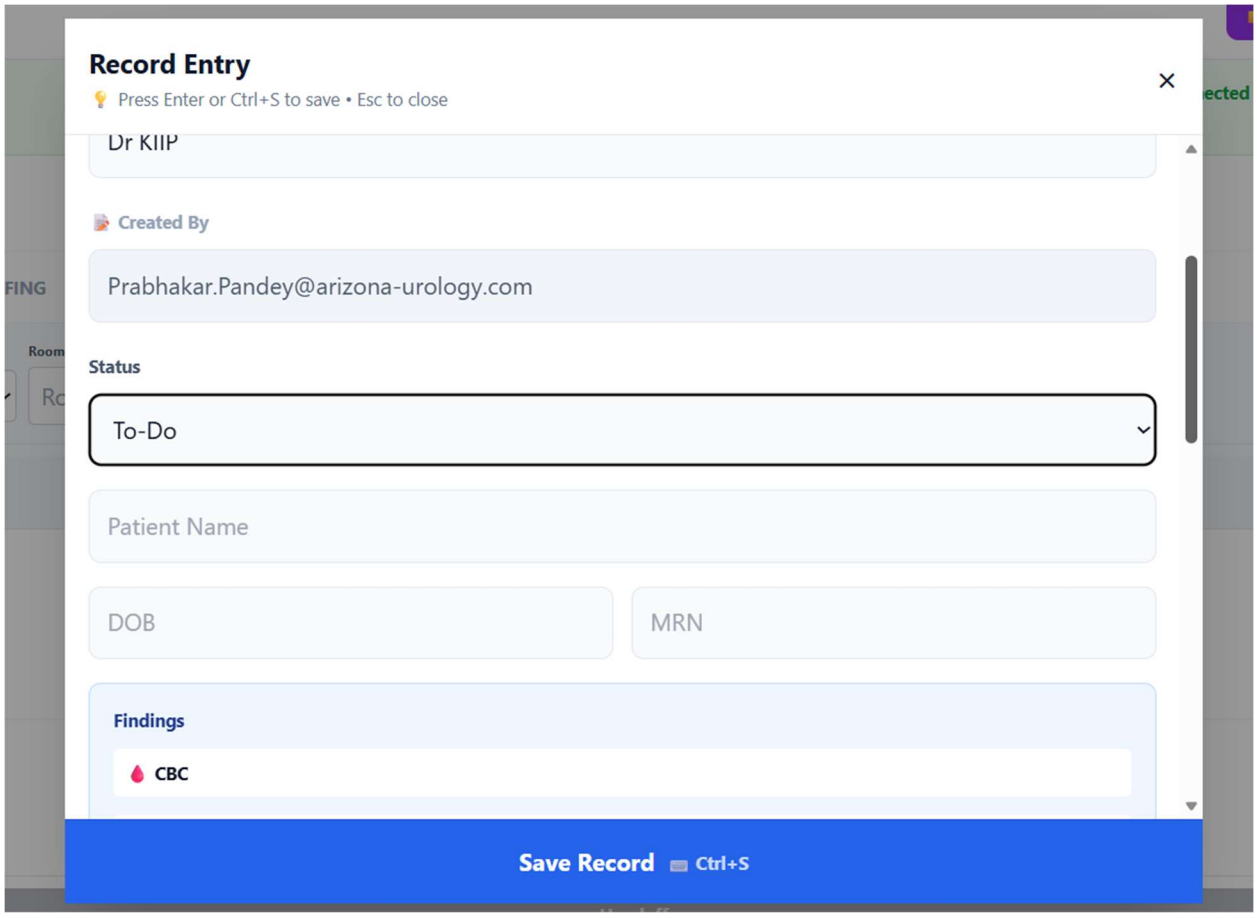
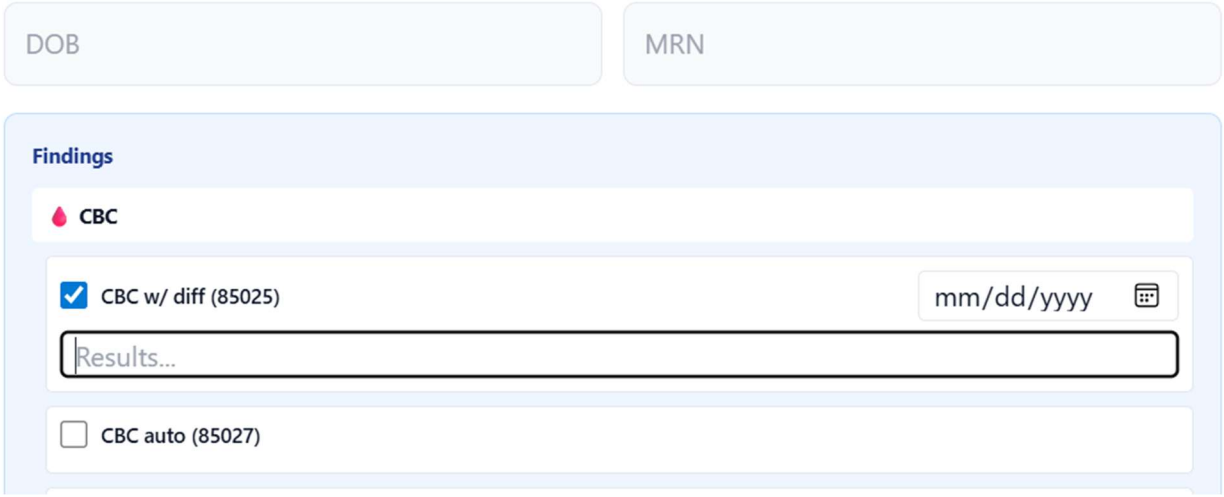


CURRENT STATUS:

To do	NEW CONSULT
In Progress	SURGICAL PATIENT (SAME DAY SURGERY)
	NEW SURGICAL PATIENT (AM ADMIT)
	FOLLOW UP (CONSULT)
Completed	FOLLOW UP (POST OP)
Post Op	TO BE DISCHARGED/SIGNED OFF



DOB: worth adding a calendar

CBC w diff:

WCC ____, Hb ____, Hct ____, Plt ____. Neutro _____% Lympho ____%, Mono ____%, Eosino ____%

Comments (if any)

Record Entry

Press Enter or Ctrl+S to save • Esc to close

2/12/1981

89889

Findings

CBC

Chemistry Panels

- [x] BMP (80048) 02/10/2026

Na ____, K _____, Cl_____, HCO3 _____, BUN _____, Creatinine_____, Glucose ____

- [ ] CMP (80053)

Microbiology

X-Ray

Save Record Ctrl+S

Record Entry

Press Enter or Ctrl+S to save • Esc to close

- [x] BMP (80048) 02/10/2026

Na ____, K _____, Cl_____, HCO3 _____, BUN _____, Creatinine_____, Glucose ____

- [ ] CMP (80053)

Microbiology

- [x] Urine culture (87086) 02/03/2026

No Growth/ Positive Growth

- [x] + organism ID (87088) mm/dd/yyyy

E coli, Staphylococcus aureus,

- [x] Blood culture (87040) mm/dd/yyyy

No Growth/Positive

- [ ] Blood fungal (87103)

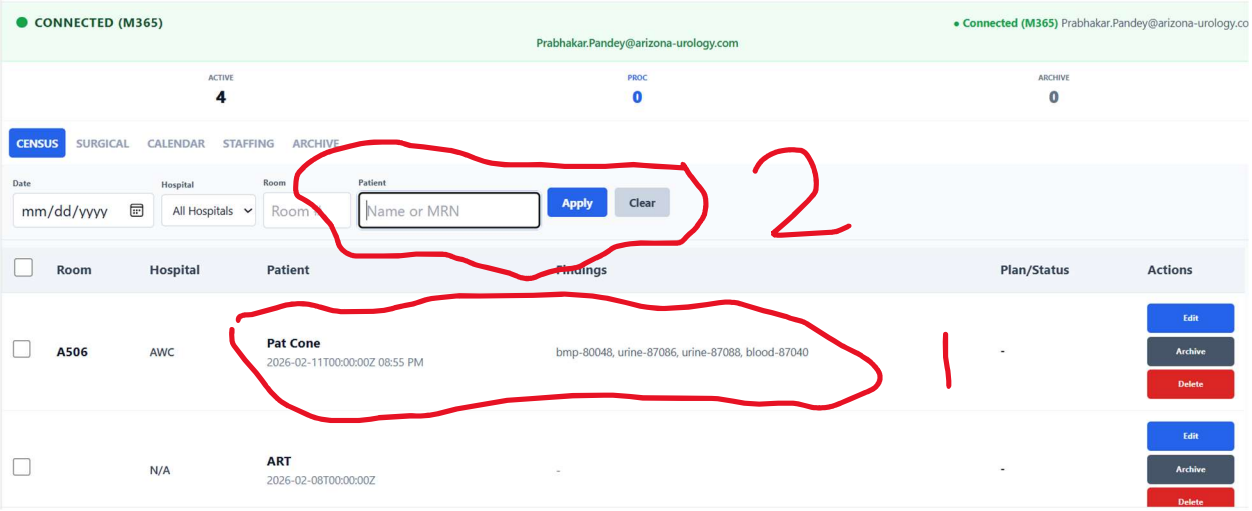
Save Record Ctrl+S

Microbiology Source:

Body Fluid	Body Tissue/Organ
Urine	Abscess Wall
Sputum	Stomach
Stool	Other
Blood	
Joint Fluid	
Other	
Other	Name of Other

Growth:

NO GROWTH	
POSITIVE GROWTH	Name of organism
Colony Count	
Resistance	
Sensitive	



1. Edit allows me to edit that particular entry. So, if I hit "Archive" will it complete my entry and not show up on this list?
2. MRN: what is the purpose of this? If I enter a MRN, will that patient record show up for editing purposes?

Radiology:

Type of Imaging	Location/Description
Xray	
Ultrasound	
CT Scan	
MRI Scan	
PET Scan	
Nuclear Scan	

Result: NORMAL/ABNORMAL

1	
2	
3	
4	
5	

Record Entry

Press Enter or Ctrl+S to save • Esc to close

MRI

Nuclear Medicine

Additional findings

Plan

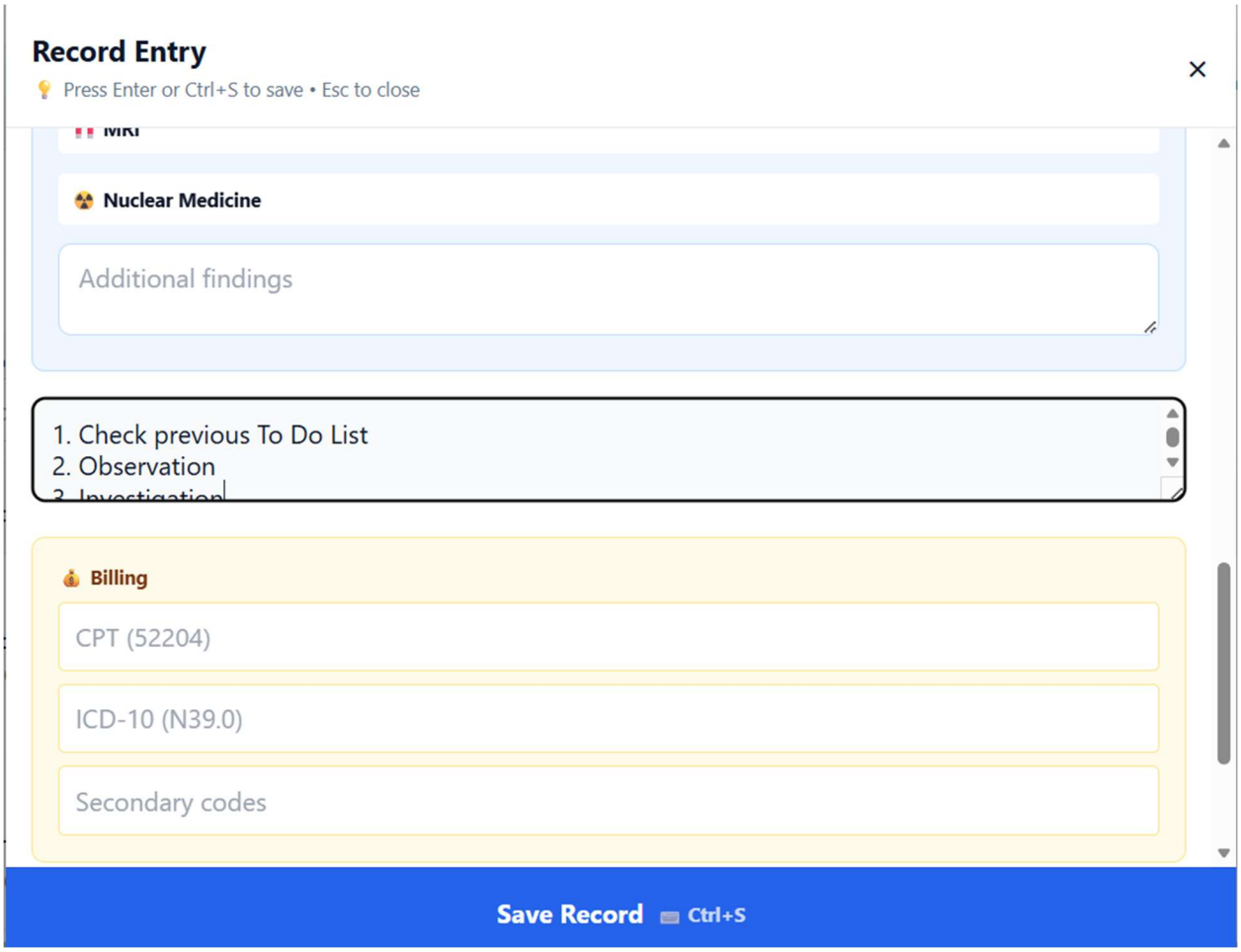
Billing

CPT (52204)

ICD-10 (N39.0)

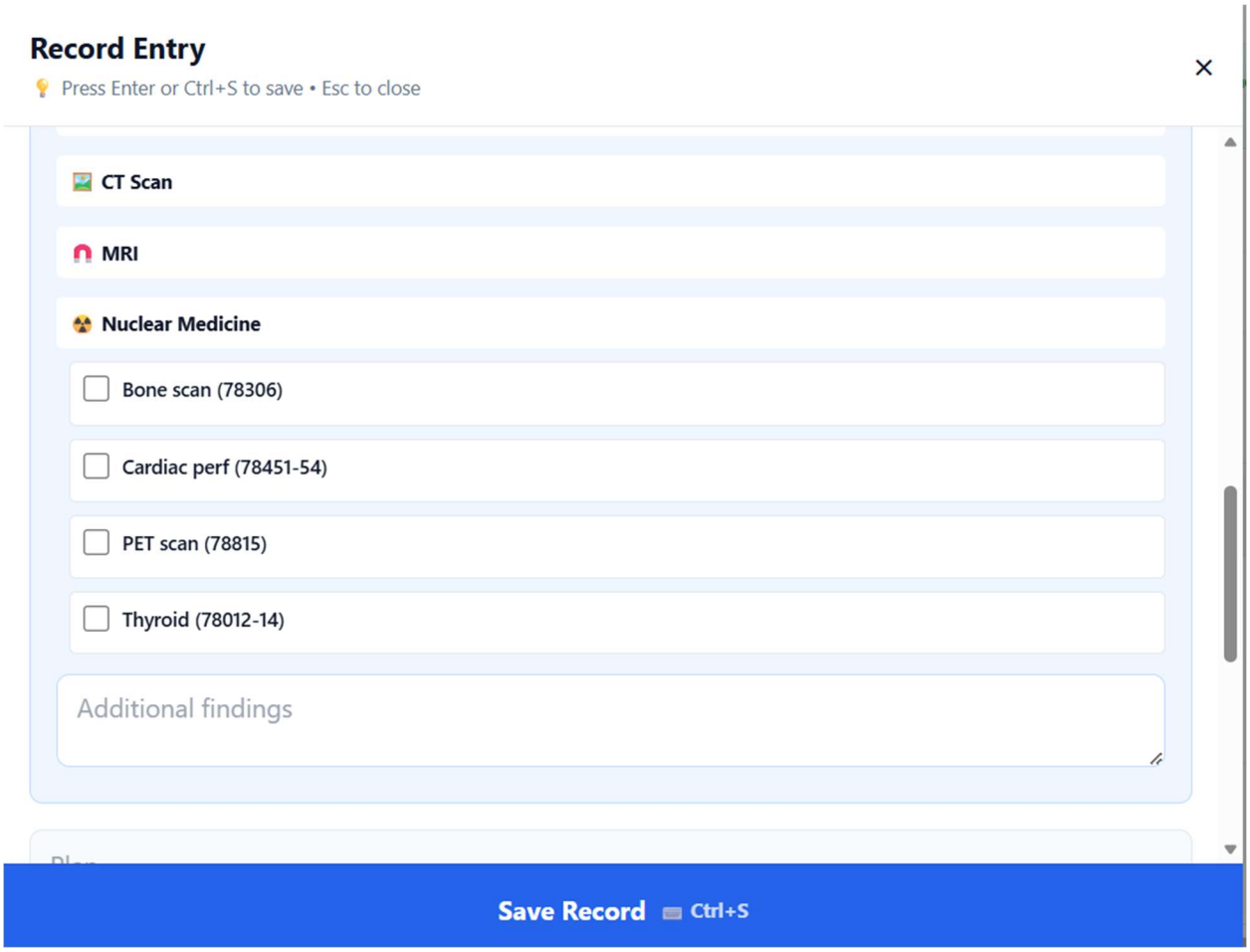
Secondary codes

Save Record Ctrl+S



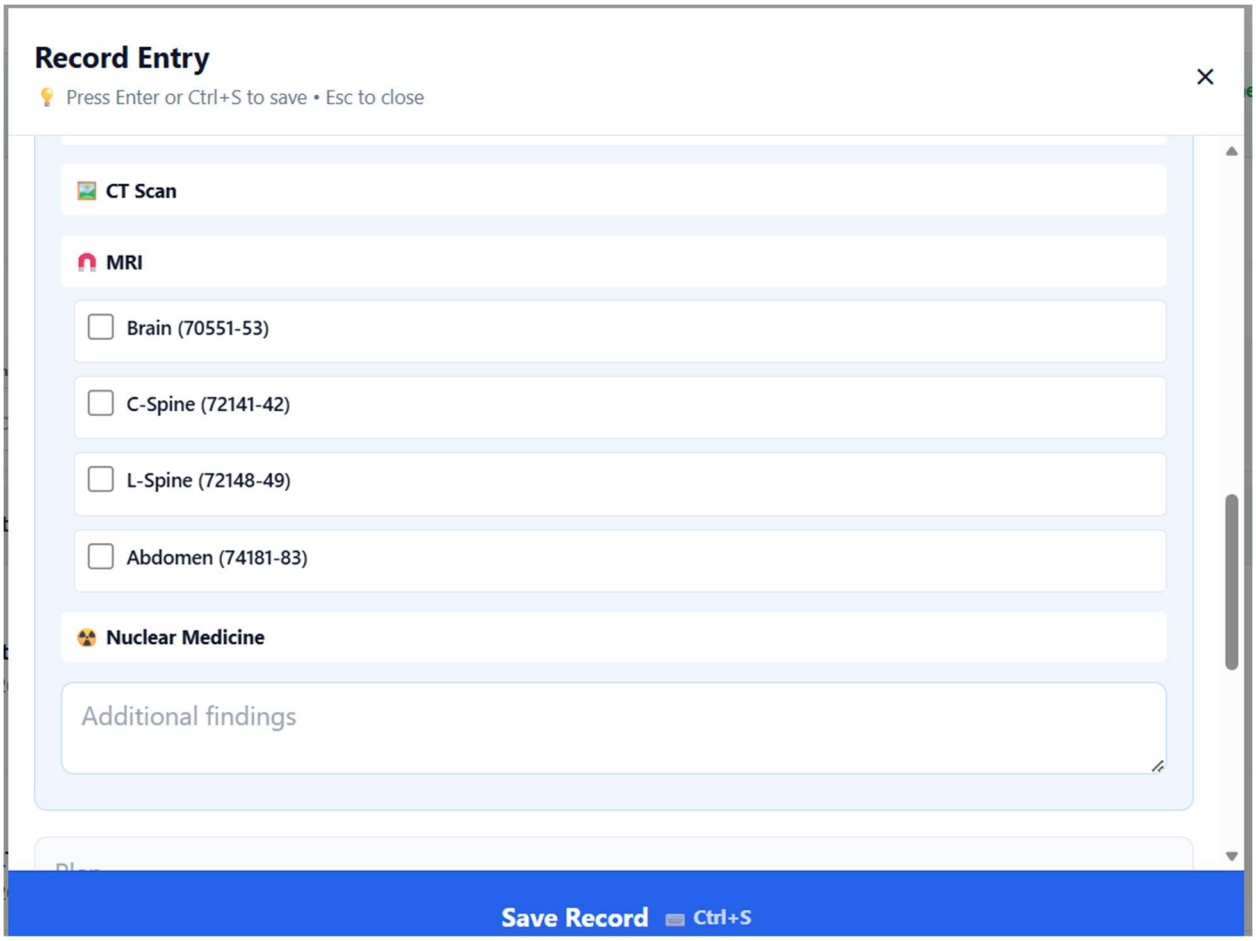
1. Check previous To Do List
2. Observation
3. Investigation
    a. Labs
    b. Radiology
    c. Consultation/2nd Opinion
    d. Special Investigations
    e. Procedure: Diagnostic
4. Pharmacotherapy
5. Physical/Behavioral Therapy
6. Surgical Procedure (Therapeutic)
7. Discharge
    a. General Instructions
        i. Activities
        ii. Diet
        iii. Restrictions, if any
    b. Pharmacotherapy
    c. Follow up Investigations
    d. Office Follow up

i. Expected time
ii. Call office if
iii. Return to ER if



Nuclear Medicine:

Name	NORMAL/ABNORMAL	Findings
Bone Scan		
LASIX RENAL SCAN		
V/Q (PULMONARY)		
CARDIAC STRESS TEST		
PSMA SCAN		



DATE	Name	NORMAL/ABNORMAL	Findings
	MRI ABD/PELVIS WO		
	MRI ABD/PELVIS W		
	MRI ABD/PELVIS		
	MRI UROGRAM		
	MRI ANGIOGRAM (RENAL)		

Record Entry

Press Enter or Ctrl+S to save • Esc to close

- Chemistry Panels
- Microbiology
- X-Ray
- Ultrasound
- CT Scan
  - [ ] Head/Brain (70450-70)
  - [ ] Chest (71250-75)
  - [ ] Abdomen (74150-78)
  - [ ] Pelvis (72192-94)
- MRI

Save Record Ctrl+S

DATE	Name	NORMAL/ABNORMAL	Findings
	CT ABD/PELVIS WO		
	CT ABD/PELVIS W		
	CT ABD/PELVIS		
	CT UROGRAM		
	CT ANGIOGRAM (RENAL)		
	CT CHEST WO		
	CT CHEST W		
	OTHER		

Record Entry

Press Enter or Ctrl+S to save • Esc to close

- Chemistry Panels
- Microbiology
- X-Ray
- Ultrasound
  - [ ] Abd complete (76700)
  - [ ] Abd limited (76705)
  - [ ] Echo (93306)
- CT Scan
  - [ ] Head/Brain (70450-70)
  - [ ] Chest (71250-75)

Save Record Ctrl+S

ULTRASOUND:

DATE	Name	NORMAL/ABNORMAL	Findings
	RENAL ULTRASOUND		
	ABDOMEN		
	RENAL DOPPLER		
	SCROTAL US		
	OTHER		

Record Entry

Press Enter or Ctrl+S to save • Esc to close

- Chemistry Panels
- Microbiology
- X-Ray
  - [ ] Chest (71045-48)
  - [ ] Spine (72020-110)
  - [ ] Extremity (73000+)
- Ultrasound
- CT Scan
- MRI
- Nuclear Medicine

Save Record Ctrl+S

X-RAY:

DATE	Name	NORMAL/ABNORMAL	Findings
	ABDOMEN FLAT/UPRIGHT		
	ABDOMEN KUB		
	ABDOMINAL SERIES		
	CHEST P/A		
	CHEST PA/LATERAL		
	OTHER		

Record Entry

Press Enter or Ctrl+S to save • Esc to close

Plan

Billing

CPT (52204)

ICD-10 (N39.0)

Secondary codes

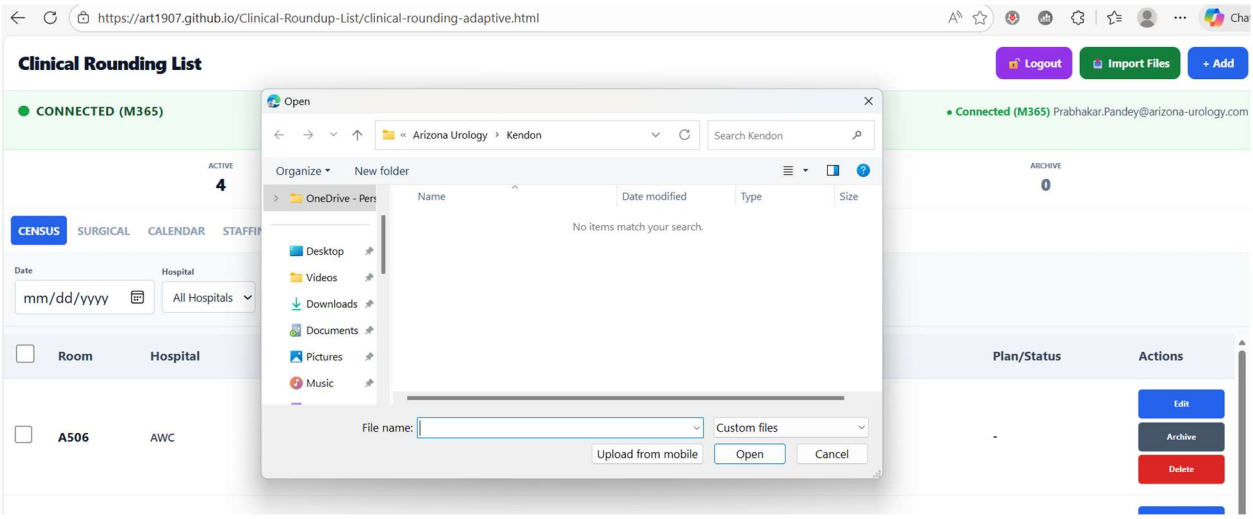
Follow-Up

Pending Tests

Save Record Ctrl+S

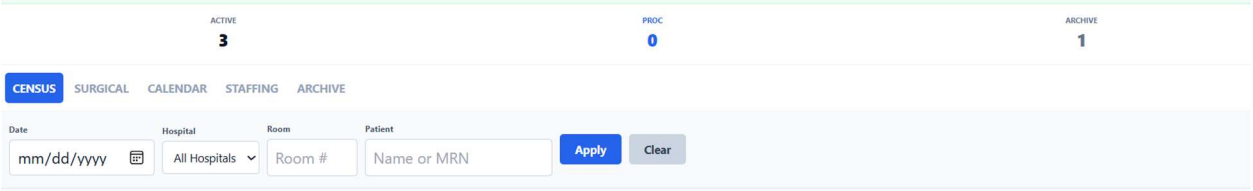
FOLLOW UP (EMAIL/FAX/PRINT)

	ACTIONS	ACTIONS
DATE		SCHEDULE
ACTIVITIES		
DIET		
RESTRICTIONS		
ALERTS		CALL/RETURN TO ER
LABS TO DO	PENDING AND/OR ADDITIONAL	
RADIOLOGY	PENDING/ADDITIONAL	
CONSULT	PENDING/ADDITIONAL	



"IMPORT FILES":

- Interesting Concept
- Patient specific data storage
- Import from:
    - EMR – PDF/IMAGE COPY ETC
    - EMAIL/TEXT/WHATSAPP ATTACHMENT SAVED ON USER'S STORAGE FOLDER
- THIS BUILDS UP CONCEPT FOR PATIENT SPECIFIC MEDANALYSE ACCOUNT ("ARCHIVE" BUILDS UP USER SPECIFIC MEDANALYSE ACCOUNT.



Can archived data be accessed? Right now it does not.

CONNECTED (M365)

Prabhakar.Pandey@arizona-urology.com

• Connected (M365) Prabhakar.Pandey@arizona-urology.

ACTIVE 4 | PROC 1 | ARCHIVE 1

CENSUS SURGICAL CALENDAR STAFFING ARCHIVE

Date: mm/dd/yyyy | Hospital: All Hospitals | Room: Room # | Patient: Name or MRN | Apply | Clear

Room	Hospital	Patient	Findings	Plan/Status	Actions
Same Day Surgery	AWC	lynn jones 2026-02-17T00:00:00Z 06:29 AM	-	Robotic Sacrocolpopexy To-Do	Edit Archive Delete
	N/A	ART 2026-02-08T00:00:00Z	-	-	Edit Archive

Same Day Surgery was created under surgical tab. I see that number of procedures is now on and “Active” number is 4. Surgery tab also shows the name of the patient.

ACTIVE 4 | PROC 1 | ARCHIVE 1

CENSUS SURGICAL CALENDAR STAFFING ARCHIVE

Date: mm/dd/yyyy | Hospital: All Hospitals | Room: Room # | Patient: Name or MRN | Apply | Clear

Room	Hospital	Patient	Findings	Plan/Status	Actions
Same Day Surgery	AWC	lynn jones 2026-02-17T00:00:00Z 06:29 AM	-	Robotic Sacrocolpopexy To-Do	Edit Archive Delete

Clinical Rounding List

Logout | Import Files | + Add

CONNECTED (M365)

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ACTIVE 4 | PROC 1 | ARCHIVE 1

CENSUS SURGICAL CALENDAR STAFFING ARCHIVE

← Prev | **February 2026** | Next →

8	9	10	11	12	13	14
15	16 us Presidents' Day	17	18	19	20	21
22	23	24	25	26	27	28

Calendar:

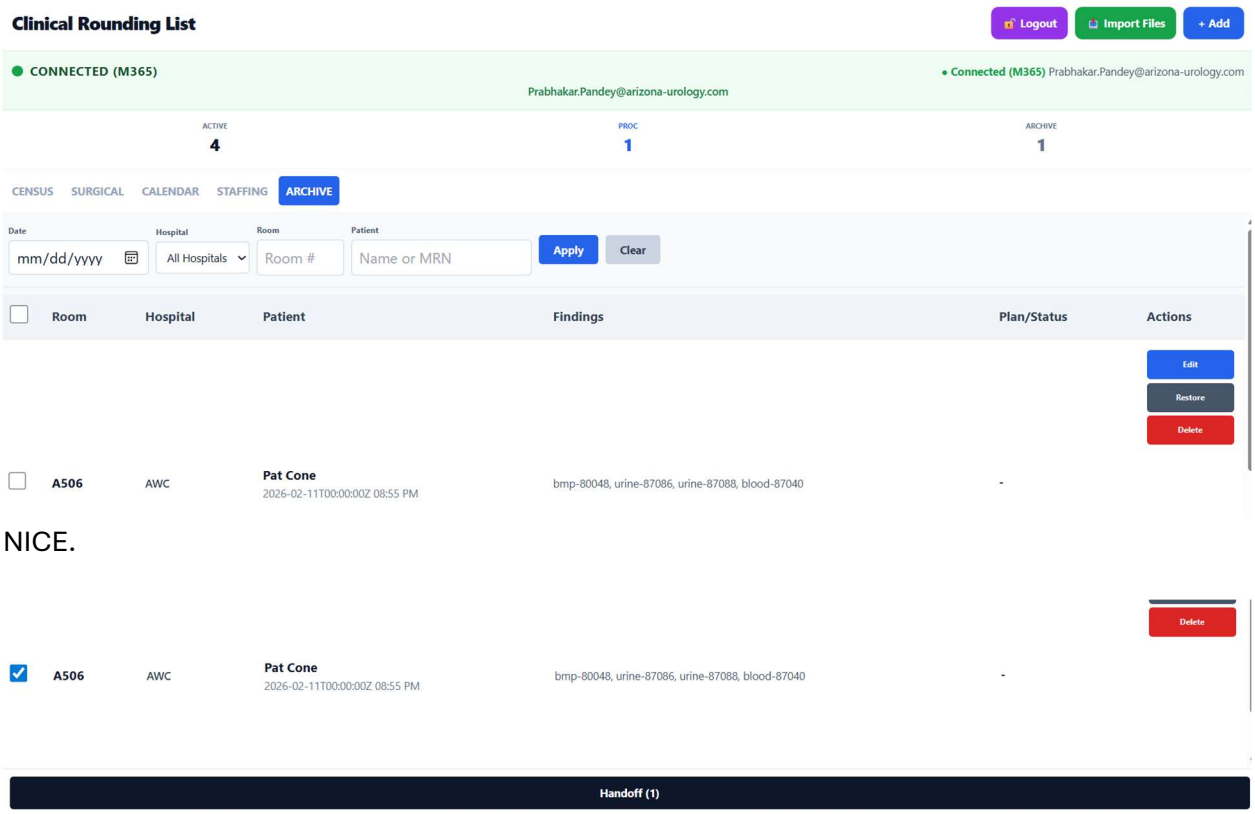
PATIENT/ADMINISTRATIVE

PATIENT

NAME	CATEGORY	ACTION	REVIEW DATE
	LAB/RADIOLOGY/MEDICATION ETC	REVIEW/ORDER	

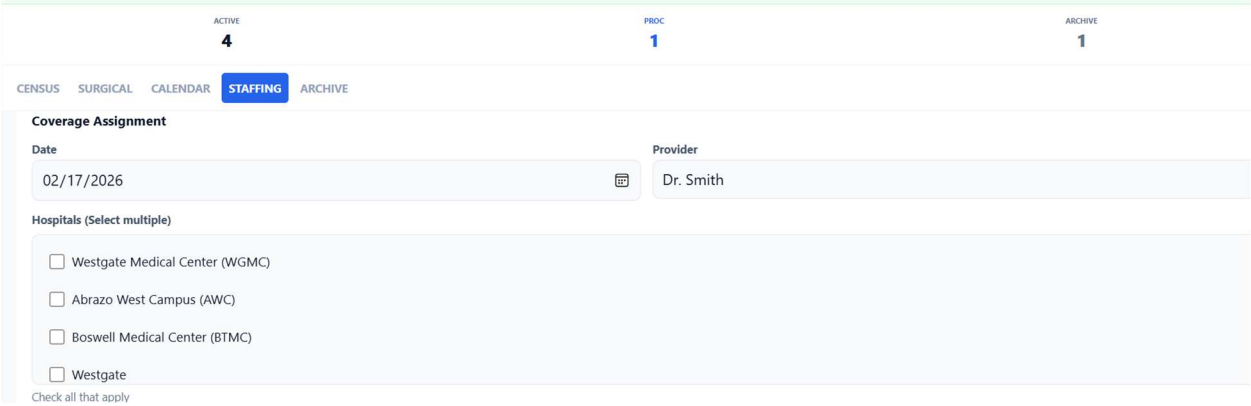
ADMINISTRATIVE

CATEGORY	ACTION	REVIEW DATE



NICE.

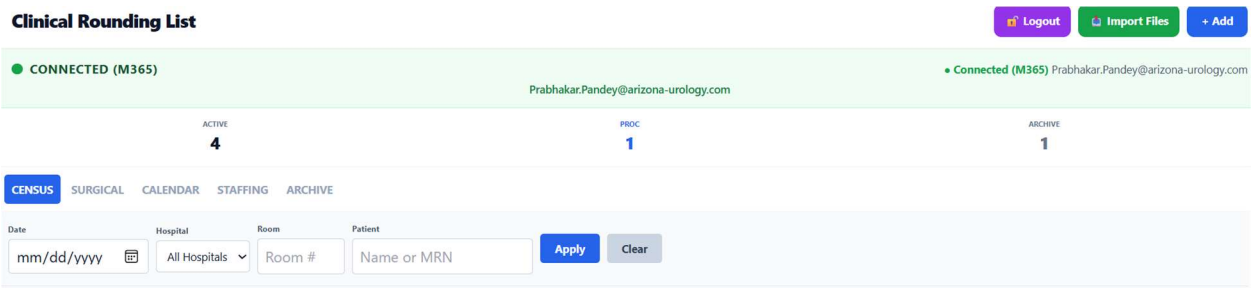
Hand Off is activated when a patient row is checked.
What does hand off mean?
Who is at the receiving end?
Does that still remain in its place within the system.



DATE	PROVIDER (PHYSICIAN)	PROVIDER (MID LEVEL)

Scheduling Surgical Procedures:
This can be elective – patient is mostly scheduled by Surgeon and Office Scheduling System. Some patients may be in hospital and be scheduled on urgent and emergency basis. Patients who are in hospital will be on rounding list – how will they be shown to be undergoing a procedure.

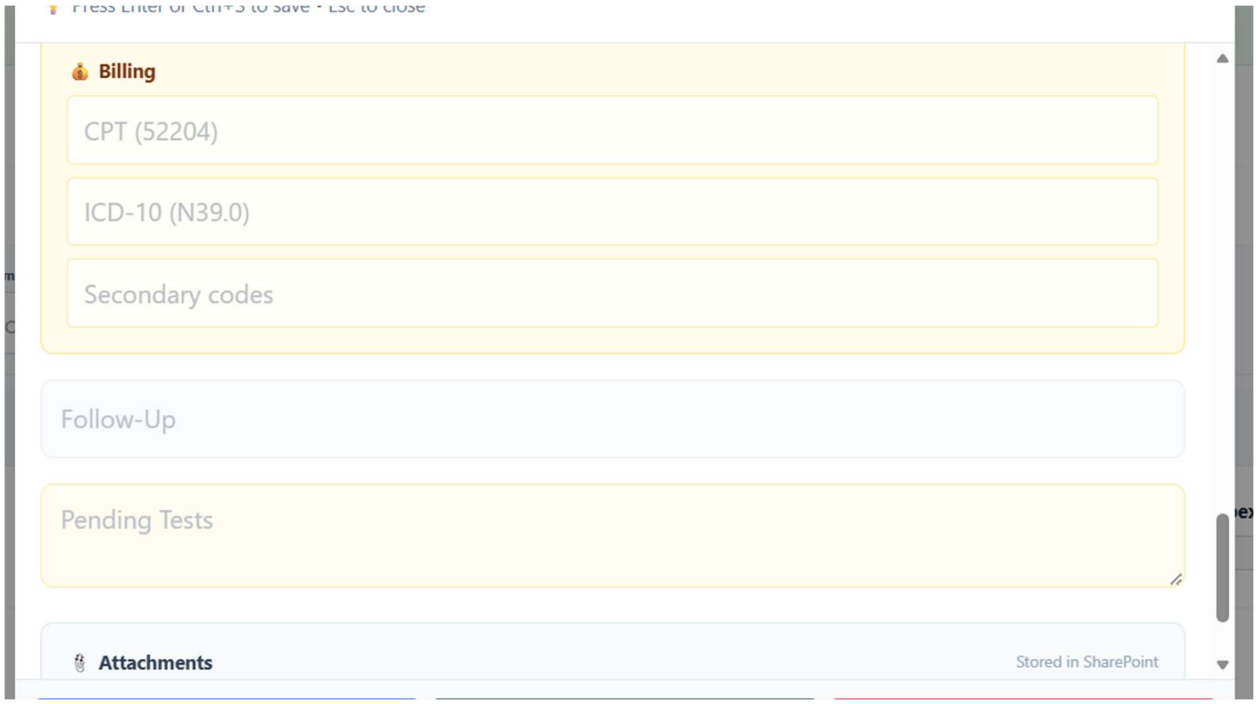
Date	
Patient	
Surgeon	
Assistant	
Procedure	
CPT	
ICD	
Required Instruments	
Required Resources (Radiology etc.)	
Path (Frozen Section)	Yes/No
Insurance Name	
Insurance Approval	Approved/Not Required
Industry Representation	Required/Not Required
Additional Comments, if any	



If we added name of the provider (physician/mid-level) in the search/filter section, would that provide specific information for that provider? How difficult would it be to do in this setting?

DATA AGGREGATION:
How difficult would it be to perform following:

- Demographic Assessment:
  - Number of patients: Daily, Weekly, Monthly
  - Geographic Distribution: ZIP code (we are not collecting that patient information directly). We do have that information embedded in factsheets.
  - Gender distribution
  - Age distribution
- Providers
  - Number of providers: Daily/Weekly/Monthly
  - ICD
  - CPT
  - Instrument Usage
  - Patient Demographics
- CPT and E&M related data
  - Daily charges or Charges at periodic intervals
    - Provider specific
    - Location specific
- ICD related data
  - Work inventory and revenue stream
  - Analysis of Charges and Revenue Stream should help understand impediments and bottlenecks
    - Can revenue stream be tapped more efficiently?
    - Inherent blocks to stream



BILLING:

DESCRIPTION	ICD	E&M	CPT
Initial Consult	N71	99253	
Cystoscopy	N71		52000